KIM INHONG

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20, Arisu-ro 97-gil, Gangdong-gu,
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1. Education

Sungkyunkwan University (SKKU)	August 25, 2025	B.A. in Sport Science (Major) B.A. in International Trade & Commerce (Double Major)

2. Work Experience

Researcher, Government-Funded Healthcare Project
(Gachon University Industry-Academic Cooperation Foundation)

March 1, 2025~February 28, 2026

Managed data collection and processing pipelines for a senior healthcare project
User retention and service operations enhancement project
Planning and design of an interactive dashboard for business promotion

3. Certificates & Awards

OPIc	August 3, 2025	Intermediate High
TOEIC	January 23, 2025	850
ADsP	June 5, 2026	
AI-POT (AI Prompt Operating)	April 30, 2026	Level 2
AIBT (AI Business Test)	April 2, 2026	Level 2
Google Analytics Certification	February 3, 2026	
Smarthome Healthcare Instructor	November 14, 2025	Level 2
Magna Cum Laude (SKKU)	August 25, 2025	
Award of Excellence as an ROTC Cadet	August 25, 2022	
Best Poster Award	November 1, 2025	Korean Society of Exercise Rehabilitation (KSER)

4. Experience & Activities

1st Cohort of the SKKU–Samsung Life Lifenology Lab	January 23, 2025	Samsung Life
Member, Human Rights & Welfare Department, 55th Student Council	December 6, 2023	SKKU
Deputy Head of External Relations, 52nd Student Council, College of Sport Science	December 6, 2022	SKKU
1st Cohort Member of “Kukdae Smarters,” Korean Sport & Olympic Committee (KSOC)	December 10, 2022	KSOC

Challenging Experiences

I have shaped my way of working through two defining experiences—one in sport, and one in the field of healthcare.

As a university floorball player, I once challenged myself to qualify for the national team. I wanted to test how far I could go in a sport I had devoted myself to for years, and the national team felt like a clear benchmark. In the weeks leading up to the tryout, however, my shooting accuracy plateaued no matter how hard I trained. At that point, I decided to rely less on intuition and more on data. I compiled feedback from coaches and teammates, recorded every training session, and analyzed over 100 hours of footage frame by frame. Through this process, I identified a pattern: the position of my chin was determining my upper-body angle, which in turn directly affected shot power and trajectory. By focusing on correcting this specific point, I improved my shooting accuracy from about 64% to 75%.

Although I ultimately fell short of making the national team, I shared the analysis and correction methods with my teammates, used data to diagnose position-specific weaknesses, and helped redesign our overall training program. The team went on to win the 2024 University National Championship. More than the result itself, this experience taught me to step away from subjective perception and examine problems through data—and to turn that insight into action others could use.

I deepened this mindset while working as a researcher on a government-funded senior digital health demonstration project. In a long-term study targeting older adults, we faced a critical challenge when participation and device adherence remained low, and data collection was frequently interrupted by extended periods of missing data. Instead of rushing into analysis, I chose to understand people first. I spent time on site, speaking one-on-one with seniors who were hesitant to join or continue the program. Many who initially declined began to open up after several conversations in a familiar setting. As trust was built, two core issues became clear: the devices were difficult to operate, and even when data was collected, participants did not know what the numbers meant for their daily lives.

To address this, I created customized guides in everyday language and combined wearable device logs with on-site measurement results to suggest personalized directions for health management. When participants looked at their own data and understood what to do that day, I could see the exact moment when numbers turned into behavior. In the end, 150 out of 224 seniors—about 67%—remained actively engaged in the program, and after SQL extraction and SPSS processing, 85% of collected records were usable for analysis, with a final retention rate of 83% among participants who had consented to the study. Through this experience with one of the least digitally familiar target groups, I learned firsthand that user context must come before data, and that data only becomes meaningful when it changes behavior.

These two experiences shaped my approach to problem-solving: I try to understand people and context first, and then use data to design actions. On the court, data changed me; in the field, it changed the way people behaved. Looking back, I was always close to the field—reviewing footage in the gym, and sitting next to seniors at the project site. I want to bring the same attitude to healthcare work: not reading metrics in isolation, but first understanding the people and environments behind them, and then using data to propose practical, executable solutions.

I hereby submit my application as detailed above.

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Strengths and Weaknesses

My biggest strength is the ability to translate on-the-ground challenges into structured data and turn them into actionable solutions.

As a member of the Student Council's Welfare & Human Rights department, I contributed to improving campus mobility for students with disabilities. Although the campus map showed ramps and elevator locations, wheelchair and crutch users repeatedly told us that the information was outdated and unreliable. I interpreted this not merely as a complaint, but as a need to ground spatial information in verified, up-to-date evidence. I interviewed students with disabilities and their assistants to understand frequently used routes and risk areas, and then walked the campus myself to verify conditions on the ground.

In parallel, I also designed and analyzed a large-scale student survey—approximately 800 responses—to prioritize library remodeling needs with quantitative support. By connecting qualitative field input with structured survey data, we were able to present clearer, evidence-based recommendations to stakeholders. This experience taught me that simply recognizing inconvenience is not enough; structuring it into data and a concrete alternative makes a fundamentally different impact.

I took the same approach in my national senior digital health project. Frequent device non-adherence led to repeated gaps in the data pipeline. To stabilize it, I extracted 69,000 raw log entries using SQL and used SPSS—including correlation and regression analysis—to filter out sensor errors and duplicate logs, achieving an 85% data usability rate for analysis. From the cleaned dataset, I worked to detect patterns that consistently appeared before participant attrition, such as nighttime device non-wear combined with a sharp decline in wearable engagement. We paired structured onboarding guides with on-site re-education, and through close field management, ultimately improved the final retention rate to 83%.

In addition, many institutional partners told us, "We have data, but we don't know how to use it." To solve this, I reorganized key indicators from their perspective and participated in designing an interactive project status dashboard prototype in Figma, tailored to how different partners needed to read project performance. This supported five positive MOU and service contract decisions. I believe this ability to read complex data and translate it into actionable, field-ready recommendations will be directly relevant whether I am supporting healthcare industry research, real-world evidence work, or digital health service decisions.

On the other hand, my tendency to strive for the "best" solution can make the initial planning phase longer than intended. As a researcher, I once spent too much time fine-tuning how to handle missing raw data and ended up finalizing a report just before the deadline. Through that experience, I realized that in a fast-moving environment, timing can be as important as accuracy. Since then, I have made it a habit to set a clear first-draft deadline before starting any task, share an 80%-complete version with my team, and refine it based on feedback. In healthcare projects—where field conditions, data quality, and stakeholder needs move quickly—I plan to set clear draft deadlines and iterate from an 80% version, rather than delaying delivery in pursuit of perfection.

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Role-Related Skills

I have built my capabilities around understanding the field through data, and driving execution through reporting and communication.

First, I have experience securing data quality and accuracy at scale. In the senior digital health demonstration project, repeated device non-adherence caused significant gaps in the data pipeline. To stabilize it, I extracted 69,000 raw log entries using SQL and applied SPSS statistical analysis to remove sensor errors and duplicate logs, achieving an 85% data usability rate for analysis. I believe this experience of detecting and correcting errors in large datasets is directly transferable to managing, preprocessing, and reporting healthcare-related data with minimal discrepancies.

Second, I have experience turning data into service operations and performance improvement. From the cleaned dataset, I identified patterns that consistently appeared before participant attrition and addressed them through field-ready interventions—user-friendly guides, on-site support, and structured onboarding—achieving a final retention rate of 83%. This process of detecting abnormal signals in data and translating them into operational strategies is relevant to any role that requires early identification of risk and practical response plans grounded in real-world evidence.

Third, I have experience in reporting, dashboarding, and stakeholder communication. To address partners' repeated concern that "we have data but don't know how to use it," I helped design a project status dashboard that allowed stakeholders to grasp key indicators at a glance and built prototype layouts using Figma. By tailoring how indicators were presented to each partner's context, I supported five positive MOU and service contract decisions. This ability to visualize complex information in a way that others can immediately understand and act on helps bridge gaps between field operations, research teams, and decision-makers.

Fourth, I have experience in digital health service planning. Through the Samsung Life × SKKU de:light project, I contributed to planning a wearable ring (fNIRS-based) and companion app, designing user flows and prototypes that connected sensor data with understandable user experience. This strengthened my ability to think about healthcare not only as data collection, but as services people can actually use.

Lastly, my academic background supports my understanding of the domain. Studying Sports Science has allowed me to understand health, movement, and physical function from an evidence-based perspective, while my International Trade studies have given me a view of market dynamics, partnerships, and organizational decision-making. Building on this foundation, I continue to strengthen my data and digital skills through certifications including ADsP (Advanced Data Analytics Semi-Professional, Korea) and hands-on work with SQL, SPSS, and analytics tools. I hope to leverage this combination to contribute wherever real-world healthcare data must become reliable insight and actionable direction.

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Future Plans

In healthcare and digital health, I aim to become someone who understands field context deeply, and uses data to make that context visible and actionable.

In my first one to two years, my priority will be to build a strong foundation in how healthcare data is collected, validated, and communicated in real-world settings. I plan to regularly work with participation, adherence, data quality, and outcome indicators—and align disparate metrics into a common framework that both field teams and decision-makers can use in discussion. Drawing on my experience of processing 69,000 logs, managing periods of missing data, and improving retention to 83%, I will focus on catching early signs of data or engagement risk and proposing responses grounded in actual user context. Rather than stopping at reporting results, I want to stay close to the field, look at the numbers together with stakeholders, and have concrete conversations about what should change next.

Over three to five years, I hope to help define practical standards for how real-world healthcare data is turned into evidence and decisions. By analyzing patterns across different project types, user groups, and operational conditions, I want to identify what drives reliable outcomes and structure those learnings into reusable frameworks—whether for research reviews, service planning, or reporting workflows. Combining my Sports Science background with my experience in digital health projects and stakeholder-facing dashboards, I aim to explain not only what the data shows, but what it means for the people and services behind it.

In the longer term, I would like to act as a bridge that keeps information flowing smoothly between the field and the organization. I believe a core responsibility in healthcare work is to translate on-the-ground voices into data and insights that teams can act on, while also interpreting organizational goals into clear, feasible plans that fit real-world conditions. Having spent most of my meaningful experiences close to the field—in sport and in senior care—I hope to stay close to end users and the teams who serve them, and to use data as a tool for stable, sustainable improvement in health outcomes.

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